

Question:
A mental health volunteer is interviewing locals as part of a community outreach program. A 46-year-old man discloses that he has felt sad for as long as he can remember. He feels as though his life is cursed and if something terrible can happen to him, it usually will. He has difficulty making decisions and feels hopeless. He also feels that he has had worsening suicidal ideations, guilt from past problems, energy, and concentration over the past 2 weeks. He is otherwise getting enough sleep and able to hold a job. Which of the following statement best describes this patient's condition?
Answer Choices:
A. The patient is at risk for double depression.
B. The patient may have symptoms of mania or psychosis.
C. The patient is likely to show anhedonia.
D. The patient likely has paranoid personality disorder.

Reasoning + Answer:

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Finding Reasoning Paths :
1 . **Early -on set depression** : The patient 's lifelong history of sadness suggests chronic , recurrent depression . Symptoms of early -on set depression include pervasive sadness , low self-esteem , pessimism , energy and concentration changes , and suicidal ideation .
2 . **Double depression** : This occurs when a person with a history of major depression (e .g . , childhood -on set) experiences subsequent episodes of major depression . The persistent negative outlook and worsening suicidal ideation over the past two weeks align with this condition .
3 . **Hypomania or mania** : While hypomania can present with mood swings and energy increases , the patient 's history of lifelong sadness and recent worsening symptoms suggests depression rather than bipolar disorder .
4 . **Personality disorder** : Paran oid personality disorder is characterized by suspicious ness , lack of trust , and sensitivity to perceived slights . The patient does not exhibit these traits .
5 . **Suicide risk** : The persistent sadness , low self-esteem , and suicidal ideation indicate a significant risk of suicide , particularly in a patient with a history of chronic depression .

Reasoning Process :
1 . **Understanding the patient 's presentation** :
- The patient 's lifelong history of sadness suggests a chronic or recurrent form of depression .
- The recent worsening of symptoms (e .g . , suicidal ideation , energy , and concentration changes) over the past two weeks indicates an acute depressive episode super imposed on his chronic condition .
- The pessim istic outlook and hope lessness are consistent with depression , as these are common features of the disorder .
- The patient 's ability to hold a job and otherwise function despite feeling hopeless suggests that his depression is severe but not totally disabling .

2 . **Evaluating the diagnosis** :
- The patient 's history of chronic sadness and the recent worsening of symptoms over two weeks strongly suggests a recurrent depressive disorder , possibly in the form of **double depression** .
- Double depression is a subtype of major depressive disorder characterized by a history of chronic depressive symptoms (e .g . , childhood or adolescence) and subsequent episodes of major depression .
- While the patient 's energy and concentration changes could theoretically suggest hypomania , his lifelong history of sadness and worsening symptoms over the past two weeks are more indicative of depression .

3 . **Assessing suicide risk** :
- The patient 's hope lessness , suicidal ideation , and recent worsening of symptoms are significant risk factors for suicide .
- The combination of chronic depression and an acute depressive episode underscores the urgent need for intervention to prevent a potentially fatal outcome .

4 . **Excluding other diagnoses** :
- The patient 's symptoms do not align with bipolar disorder , as there is no evidence of hypomanic or manic episodes .
- The patient does not exhibit the pervasive distrust and paranoia associated with paranoid personality disorder , which is unrelated to his current presentation .
- There is no mention of psychotic symptoms , mania , or hypomania that would suggest other mood disorders .

Conclusion :
The patient is at risk for double depression , characterized by a history of early -on set depression and recent worsening of symptoms , which may lead to suicide . Treatment should prioritize addressing the suicidal ideation and managing the depressive episode , with potential consideration of psychotherapy and pharmacotherapy .
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<answer>
The patient is at risk for double depression .
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